

Uncertainty, Conflict Resolution, and Abstention Policy

Deterministic evidence states, hard stops, escalation, and safest-next-action rules

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Owner: Patient Safety Officer
Approved by: Fictional Clinical Governance Committee
Classification: Training / synthetic corpus

Document control and retrieval metadata

Field	Value
Canonical title	Uncertainty, Conflict Resolution, and Abstention Policy
Document ID	NB-SAF-403
Keywords	uncertainty, conflict, abstention, evidence, missing data, safest action, escalation
Authority	Patient Safety Officer
Applicability	Training simulations only
Relationship	Standalone extension; existing corpus documents remain unchanged

Retrieval rule

Always retrieve when data is missing, stale, inconsistent, unsupported, or produced by a failed tool; also retrieve before assigning a final review status.

1. Governing principles

The system must prefer a safe, explicit stop over an unsupported completion. Model fluency, self-reported confidence, or agreement between multiple model calls is not evidence of clinical correctness.

- Missing is not normal; unknown is not negative; absence of a retrieved warning is not proof of safety.
- Never fill missing dose, unit, route, frequency, duration, indication, authorization, allergy status, or patient identity from model memory.
- Separate facts, authorized orders, policy requirements, external evidence, and model interpretation.
- Every non-final outcome must state the safest next action, owner, urgency, and information needed to proceed.

2. Review-status state machine

Status	Permitted meaning	Required behavior
FINAL_AUTHORIZED	All hard gates pass and a current simulated clinician-signed order exists	Return complete recommendation and plan with authorization/citations
DRAFT_FOR_CLINICIAN_REVIEW	Evidence supports a draft but authorization or noncritical review remains	Label every proposed action as unapproved
INSUFFICIENT_EVIDENCE	Material information is missing, stale, or unsupported	Withhold affected recommendation; request exact missing evidence
CONFLICT_REQUIRES_REVIEW	Two material facts/orders/sources disagree or safety flag conflicts with order	Do not choose; route to appropriate clinician/pharmacist/governance owner
URGENT_CLINICAL_REVIEW	Potentially important risk needs prompt human assessment	Bypass routine queue and use defined urgent route
EMERGENCY_ESCALATION	Configured emergency presentation or explicit emergency flag	Stop routine workflow and display emergency pathway
SYSTEM_UNAVAILABLE	Identity, access, or required dependency prevents safe processing	Return no recommendation dependent on unavailable function

3. Deterministic evidence grade

Grade	Criteria
HIGH	Verified identity; complete/reconciled current data; current authorized order; current provenance-checked evidence; no unresolved conflict; claims/citations validated
MODERATE	No hard stop, but a noncritical gap or limited source breadth remains; final status requires clinician rule
LOW	Material uncertainty exists; only factual summary and next steps allowed

Grade	Criteria
INSUFFICIENT	Any hard gate fails; no final medication recommendation allowed

The grade is calculated by application rules. The LLM may explain the grade but may not select or raise it.

4. Conflict and missing-data matrix

Condition	Status	Safest next action
Patient identifiers disagree	SYSTEM_UNAVAILABLE	Stop; identity verification by authorized staff
Medication dose/unit/formulation differs across sources	CONFLICT_REQUIRES_REVIEW	Pharmacist reconciliation and prescriber confirmation
Signed order conflicts with allergy/safety hold	CONFLICT_REQUIRES_REVIEW or URGENT	Same-day prescriber/pharmacist review; no automatic execution
Required monitoring missing or stale	INSUFFICIENT_EVIDENCE	Book test; named clinician decides interim supply/action
Result lacks unit or compatible context	INSUFFICIENT_EVIDENCE	Obtain corrected result; no numeric comparison
Preliminary result only	DRAFT or INSUFFICIENT	Wait for final unless clinician directs urgent action
Two current policies conflict	CONFLICT_REQUIRES_REVIEW	Knowledge-governance plus clinical review
Required external tool fails	SYSTEM_UNAVAILABLE or safe partial review	Disclose failure; do not use model memory as substitute
Citation does not entail claim	INSUFFICIENT_EVIDENCE	Remove claim or retrieve supporting evidence
Model contradicts structured record	CONFLICT_REQUIRES_REVIEW	Reject model output; preserve validated record and audit event

5. Source precedence and non-resolution

- Verified patient identity and current patient record provide patient facts.
- A current signed patient-specific order provides authorization details.
- Allergy, safety-hold, critical-result, and contraindication flags can block execution and require review.
- Current approved policies constrain workflow.
- Current approved labels/guidelines support explanations.
- Model synthesis is always lowest authority.
- Precedence never permits silent resolution of a safety-critical disagreement.

6. Uncertainty disclosure template

Status: [state]. Known: [verified facts with timestamps]. Missing/stale: [fields]. Conflict: [sources and exact disagreement]. Effect: [which conclusion cannot be made]. Safest next action: [action, owner, urgency]. Evidence grade: [deterministic grade].

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